agilus diagnostics

PATIENT NAME : SARITA V GAUTAM **REF. DOCTOR :** SELF

CODE/NAME & ADDRESS : C000132123 HCL AVITAS PVT LTD 806 SIDDARTHA, 96 NEHRU PLACE,SOUTH DELHI NEW DELHI 110019 9910998037	ACCESSION NO : **0062YI001258** PATIENT ID : H1.HH-141524 CLIENT PATIENT ID: H1.HH-141524 ABHA NO :	AGE/SEX :61 Years Female DRAWN :19/09/2025 08:09:39 RECEIVED : 19/09/2025 08:55:15 REPORTED :19/09/2025 19:54:56

Test Report Status **Final** | **Results** | **Biological Reference Interval** | **Units**

HAEMATOLOGY - CBC

NEWGEN - HOME VISIT PACKAGE

BLOOD COUNTS,EDTA WHOLE BLOOD

Test	Results	Biological Reference Interval	Units
HEMOGLOBIN (HB) METHOD : CYANMETHEMOGLOBIN METHOD	14.2	12.0 - 15.0	g/dL
RED BLOOD CELL (RBC) COUNT METHOD : IMPEDANCE	4.76	3.8 - 4.8	mil/µL
WHITE BLOOD CELL (WBC) COUNT METHOD : IMPEDANCE	6.63	4.0 - 10.0	thou/µL
PLATELET COUNT METHOD : IMPEDANCE	273	150 - 410	thou/µL

RBC AND PLATELET INDICES

Test	Results	Biological Reference Interval	Units
HEMATOCRIT (PCV) METHOD : CALCULATED	41.6	36 - 46	%
MEAN CORPUSCULAR VOLUME (MCV) METHOD : CELL COUNTER	87.4	83 - 101	fL
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	29.9	27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION (MCHC) METHOD : CALCULATED PARAMETER	34.2	31.5 - 34.5	g/dL
RED CELL DISTRIBUTION WIDTH (RDW) METHOD : CALCULATED	**15.5 High**	11.6 - 14.0	%
MEAN PLATELET VOLUME (MPV) METHOD : CALCULATED PARAMETER	7.7	6.8 - 10.9	fL

WBC DIFFERENTIAL COUNT

Test	Results	Biological Reference Interval	Units
NEUTROPHILS METHOD : IMPEDANCE / MICROSCOPY	54	40 - 80	%
LYMPHOCYTES METHOD : IMPEDANCE / MICROSCOPY	35	20 - 40	%
MONOCYTES METHOD : IMPEDANCE / MICROSCOPY	4	2 - 10	%
EOSINOPHILS	**7 High**	1 - 6	%

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ULR No.775000014359781-0062

PATIENT NAME : SARITA V GAUTAM		**REF. DOCTOR :** SELF
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METHOD : IMPEDANCE / MICROSCOPY			
BASOPHILS	0	0 - 2	%
METHOD : MICROSCOPIC EXAMINATION			
ABSOLUTE NEUTROPHIL COUNT	3.58	2.0 - 7.0	thou/µL
METHOD : CALCULATED PARAMETER			
ABSOLUTE LYMPHOCYTE COUNT	2.32	1 - 3	thou/µL
METHOD : CALCULATED PARAMETER			
ABSOLUTE MONOCYTE COUNT	0.27	0.20 - 1.00	thou/µL
METHOD : CALCULATED PARAMETER			
ABSOLUTE EOSINOPHIL COUNT	0.46	0.02 - 0.50	thou/µL
METHOD : CALCULATED PARAMETER			
ABSOLUTE BASOPHIL COUNT	0	0.00 - 0.10	thou/µL
METHOD : CALCULATED PARAMETER			

PS(PERIPHERAL SMEAR EXAM,EDTA WHOLE BLOOD

RBC	PREDOMINANTLY NORMOCYTIC NORMOCHROMIC		
METHOD : STAINING/ MICROSCOPY			
WBC	NORMAL MORPHOLOGY		
METHOD : STAINING/ MICROSCOPY			
PLATELETS	NORMAL IN NUMBER AND MORPHOLOGY.		
METHOD : STAINING/ MICROSCOPY			

ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD

E.S.R	**60 High**	0 - 20	mm at 1 hr
METHOD : MODIFIED WESTERGREN			

Interpretation(s)
BLOOD COUNTS,EDTA WHOLE BLOOD-The cell morphology is well preserved for 24hrs. However after 24-48 hrs a progressive increase in MCV and HCT is observed leading to a decrease in MCHC. A direct smear is recommended for an accurate differential count and for examination of RBC morphology.
RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.
WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease.

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(Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504
This ratio element is a calculated parameter and out of NABL scope.
ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD-**TEST DESCRIPTION** :- Erythrocyte sedimentation rate (ESR) is a test that indirectly measures the degree of inflammation present in the body. The test actually measures the rate of fall (sedimentation) of erythrocytes in a sample of blood that has been placed into a tall, thin, vertical tube. Results are reported as the millimetres of clear fluid (plasma) that are present at the top portion of the tube after one hour. Nowadays fully automated instruments are available to measure ESR.
- ESR is not diagnostic it is a non-specific test that may be elevated in a number of different conditions. It provides general information about the presence of an inflammatory condition.CRP is superior to ESR because it is more sensitive and reflects a more rapid change.
TEST INTERPRETATION : **Increase** in: Infections, Vasculities, Inflammatory arthritis, Renal disease, Anemia, Malignancies and plasma cell dyscrasias, Acute allergy Tissue injury, Pregnancy, Estrogen medication, Aging.
Finding a very accelerated ESR**(>100 mm/hour)** in patients with ill-defined symptoms directs the physician to search for a systemic disease (Paraproteinemias, Disseminated malignancies, connective tissue disease, severe infections such as bacterial endocarditis).
In pregnancy BRI in first trimester is 0-48 mm/hr(62 if anemic) and in second trimester (0-70 mm /hr(95 if anemic). ESR returns to normal 4th week post partum.
Decreased in: Polycythermia vera, Sickle cell anemia
LIMITATIONS : **False elevated** ESR : Increased fibrinogen, Drugs(Vitamin A, Dextran etc), Hypercholesterolemia
False Decreased : Poikilocytosis,(SickleCells,spherocytes),Microcytosis, Low fibrinogen, Very high WBC counts, Drugs(Quinine, salicylates)
REFERENCE : Nathan and Oski's Haematology of Infancy and Childhood, 5th edition 2. Paediatric reference intervals. AACC Press, 7th edition. Edited by S. Soldin 3. The reference for the adult reference range is "Practical Haematology by Dacie and Lewis,10th edition.

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HAEMATOLOGY

NEWGEN - HOME VISIT PACKAGE

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD

Test	Results	Biological Reference Interval	Units
HBA1C METHOD : HPLC	5.5	Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 Therapeutic goals: < 7.0 Action suggested : > 8.0 (ADA Guideline 2021)	%
ESTIMATED AVERAGE GLUCOSE(EAG)	111.2	< 116.0	mg/dL

Interpretation(s)
GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-**Used For**:

1. Evaluating the long-term control of blood glucose concentrations in diabetic patients.
2. Diagnosing diabetes.
3. Identifying patients at increased risk for diabetes (prediabetes).

The ADA recommends measurement of HbA1c (typically 3-4 times per year for type 1 and poorly controlled type 2 diabetic patients, and 2 times per year for well-controlled type 2 diabetic patients) to determine whether a patients metabolic control has remained continuously within the target range.

1. eAG (Estimated average glucose) converts percentage HbA1c to md/dl, to compare blood glucose levels.
2. eAG gives an evaluation of blood glucose levels for the last couple of months.
3. eAG is calculated as eAG (mg/dl) = 28.7 * HbA1c - 46.7

HbA1c Estimation can get affected due to :

1. Shortened Erythrocyte survival : Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g. recovery from acute blood loss,hemolytic anemia) will falsely lower HbA1c test results.Fructosamine is recommended in these patients which indicates diabetes control over 15 days.
2. Vitamin C & E are reported to falsely lower test results.(possibly by inhibiting glycation of hemoglobin.
3. Iron deficiency anemia is reported to increase test results. Hypertriglyceridemia,uremia, hyperbilirubinemia, chronic alcoholism,chronic ingestion of salicylates & opiates addiction are reported to interfere with some assay methods,falsely increasing results.
4. Interference of hemoglobinopathies in HbA1c estimation is seen in

a) Homozygous hemoglobinopathy. Fructosamine is recommended for testing of HbA1c.
b) Heterozygous state detected (D10 is corrected for HbS & HbC trait.)
c) HbF > 25% on alternate paltform (Boronate affinity chromatography) is recommended for testing of HbA1c.Abnormal Hemoglobin electrophoresis (HPLC method) is recommended for detecting a hemoglobinopathy

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DIAGNOSTIC REPORT

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BIOCHEMISTRY

NEWGEN - HOME VISIT PACKAGE

GLUCOSE FASTING,FLUORIDE PLASMA

Test	Results	Biological Reference Interval	Units
FBS (FASTING BLOOD SUGAR) METHOD : HEXOKINASE	97	(Normal <100,Impaired fasting glucose:100 to 125,Diabetes mellitus:>=126(on more than 1 occasion)(ADA guidelines 2024)	mg/dL

LIVER FUNCTION PROFILE, SERUM

Test	Results	Biological Reference Interval	Units
BILIRUBIN, TOTAL METHOD : DIAZONIUM ION, BLANKED (ROCHE)	0.47	0.0 - 1.20	mg/dL
BILIRUBIN, DIRECT METHOD : DIAZONIUM ION, BLANKED (ROCHE)	0.16	< or = 0.3	mg/dL
BILIRUBIN, INDIRECT METHOD : CALCULATED PARAMETER	0.31	0.00 - 0.90	mg/dL
TOTAL PROTEIN METHOD : BIURET,SERUM BLANK,ENDPOINT	7.5	6.4 - 8.3	g/dL
ALBUMIN METHOD : BROMOCRESOL PURPLE	4.3	3.97 - 4.94	g/dL
GLOBULIN METHOD : CALCULATED PARAMETER	3.2	2.0 - 4.0	g/dL
ALBUMIN/GLOBULIN RATIO METHOD : CALCULATED PARAMETER	1.3	1.0 - 2.0	RATIO
ASPARTATE AMINOTRANSFERASE(AST/SGOT) METHOD : IFCC WITH PYRIDOXAL 5 PHOSPHATE	18	0 - 32	U/L
ALANINE AMINOTRANSFERASE (ALT/SGPT) METHOD : UV WITH P5P-IFCC	19	0 - 33	U/L
ALKALINE PHOSPHATASE METHOD : PNPP, AMP BUFFER-IFCC	**116 High**	35 - 104	U/L
GAMMA GLUTAMYL TRANSFERASE (GGT) METHOD : G-GLUTAMYL-CARBOXY-NITROANILIDE-IFCC	32	5 - 36	U/L
LACTATE DEHYDROGENASE METHOD : L TO P, IFCC	165	135 - 214	U/L

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BLOOD UREA NITROGEN (BUN), SERUM			
BLOOD UREA NITROGEN METHOD : UREASE - UV	10	8 - 23	mg/dL
CREATININE, SERUM			
CREATININE METHOD : ALKALINE PICRATE	0.64	0.60 - 1.20	mg/dL
URIC ACID, SERUM			
URIC ACID METHOD : URICASE, COLORIMETRIC	4.5	2.4 - 5.7	mg/dL
CALCIUM, SERUM			
CALCIUM METHOD : BAPTA	9.7	8.8 - 10.2	mg/dL
IRON, SERUM			
IRON METHOD : FERENE	71	37 - 145	µg/dL
PHOSPHORUS, SERUM			
PHOSPHORUS METHOD : PHOSPHOMOLYBDATE METHOD	3.7	2.7 - 4.5	mg/dL

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Interpretation(s)
GLUCOSE FASTING,FLUORIDE PLASMA-**TEST DESCRIPTION**
Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and sothat no glucose is excreted in the urine.
Increased in:Diabetes mellitus, Cushing' s syndrome (10 – 15%), chronic pancreatitis (30%). Drugs:corticosteroids,phenytoin, estrogen, thiazides.
Decreased in :Pancreatic islet cell disease with increased insulin,insulinoma,adrenocortical insufficiency,hypopituitarism,diffuse liver disease, malignancy(adrenocortical,stomach,fibrosarcoma),infant of a diabetic mother,enzyme deficiency diseases(e.g.galactosemia),Drugs-insulin,ethanol,propranolol sulfonylureas,tolbutamide,and other oral hypoglycemic agents.
NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values),there is wide fluctuation within individuals.Thus, glycosylated hemoglobin(HbA1c) levels are favored to monitor glycemic control.
High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment,Renal Glyosuria,Glycaemic index & response to food consumed,Alimentary Hypoglycemia,Increased insulin response & sensitivity etc.
LIVER FUNCTION PROFILE, SERUM-
Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice.**Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors &Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.
AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver,liver cancer,kidney failure,hemolytic anemia,pancreatitis,hemochromatosis. AST levels may also increase after a heart attack or strenuous activity.ALT test measures the amount of this enzyme in the blood.ALT is found mainly in the liver, but also in smaller amounts in the kidneys,heart,muscles, and pancreas.It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health.AST levels increase during acute hepatitis,sometimes due to a viral infection,ischemia to the liver,chronic hepatitis,obstruction of bile ducts,cirrhosis.
ALP is a protein found in almost all body tissues.Tissues with higher amounts of ALP include the liver,bile ducts and bone.Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Pagets disease,Rickets,Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia,Malnutrition,Protein deficiency,Wilsons disease.
GGT is an enzyme found in cell membranes of many tissues mainly in the liver,kidney and pancreas.It is also found in other tissues including intestine,spleen,heart, brain and seminal vesicles.The highest concentration is in the kidney,but the liver is considered the source of normal enzyme activity.Serum GGT has been widely used as an index of liver dysfunction.Elevated serum GGT activity can be found in diseases of the liver,biliary system and pancreas.Conditions that increase serum GGT are obstructive liver disease,high alcohol consumption and use of enzyme-inducing drugs etc.
Total Protein also known as total protein,is a biochemical test for measuring the total amount of protein in serum.Protein in the plasma is made up of albumin and globulin.Higher-than-normal levels may be due to:Chronic inflammation or infection,including HIV and hepatitis B or C,Multiple myeloma,Waldenstroms disease.Lower-than-normal levels may be due to: Agammaglobulinemia,Bleeding (hemorrhage),Burns,Glomerulonephritis,Liver disease, Malabsorption,Malnutrition,Nephrotic syndrome,Protein-losing enteropathy etc.
Albumin is the most abundant protein in human blood plasma.It is produced in the liver.Albumin constitutes about half of the blood serum protein.Low blood albumin levels (hypoalbuminemia) can be caused by:Liver disease like cirrhosis of the liver, nephrotic syndrome,protein-losing enteropathy,Burns,hemodilution,increased vascular permeability or decreased lymphatic clearance,malnutrition and wasting etc
BLOOD UREA NITROGEN (BUN), SERUM-**Causes of Increased** levels include Pre renal (High protein diet, Increased protein catabolism, GI haemorrhage, Cortisol, Dehydration, CHF Renal), Renal Failure, Post Renal (Malignancy, Nephrolithiasis, Prostatism)
Causes of decreased level include Liver disease, SIADH.
CREATININE, SERUM-**Higher than normal level may be due to:**
• Blockage in the urinary tract, Kidney problems, such as kidney damage or failure, infection, or reduced blood flow, Loss of body fluid (dehydration), Muscle problems, such as breakdown of muscle fibers, Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia)
Lower than normal level may be due to:• Myasthenia Gravis, Muscuophy
URIC ACID, SERUM-**Causes of Increased levels:**-Dietary(High Protein Intake,Prolonged Fasting,Rapid weight loss),Gout,Lesch nyhan syndrome,Type 2 DM,Metabolic syndrome **Causes of decreased levels**-Low Zinc intake,OCP,Multiple Sclerosis
CALCIUM, SERUM-**Common causes of decreased value of calcium (hypocalcemia)** are chronic renal failure, hypomagnesemia and hypoalbuminemia.
Hypercalcemia (increased value of calcium) can be caused by increased intestinal absorption (vitamin D intoxication), increased skeletal reabsorption (immobilization), or a combination of mechanisms (primary hyperparathyroidism). Primary hyperparathyroidism and malignancy accounts for 90-95% of all cases of hypercalcemia.
Values of total calcium is affected by serum proteins, particularly albumin thus, latter's value should be taken into account when interpreting serum calcium levels. The following regression equation may be helpful.
Corrected total calcium (mg/dl)= total calcium (mg/dl) + 0.8 (4- albumin [g/dl])*
because regression equations vary among group of patients in different physiological and pathological conditions, mathematical corrections are only approximations. The possible mathematical corrections should be replaced by direct determination of free calcium by ISE. A common and important source of preanalytical error in the measurement of calcium is prolonged torniquet application during sampling. Thus, this along with fist clenching should be avoided before phlebotomy.
IRON, SERUM-Serum iron test is useful for etio- morphological diagnosis of anemias, in hemochromatosis, in hemosiderosis and in acute iron toxicity. Serum iron is recommended to be correlated with Total Iron Binding Capacity (TIBC) for evaluation of iron deficiency.

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BIOCHEMISTRY - LIPID

NEWGEN - HOME VISIT PACKAGE

LIPID PROFILE WITH CALCULATED LDL,SERUM

Test	Results	Biological Reference Interval	Units
CHOLESTEROL, TOTAL	**270 High**	< 200 Desirable 200 - 239 Borderline High >/= 240 High	mg/dL
METHOD : CHOLESTEROL OXIDASE, ESTERASE,PEROXIDASE			
TRIGLYCERIDES	113	< 150 Normal 150 - 199 Borderline High 200 - 499 High >/=500 Very High	mg/dL
METHOD : ENZYMATIC, END POINT			
HDL CHOLESTEROL	40	< 40 Low >/=60 High	mg/dL
METHOD : DIRECT MEASURE POLYMER-POLYANION			
CHOLESTEROL LDL	**207 High**	< 100 Optimal 100 - 129 Near optimal/ above optimal 130 - 159 Borderline High 160 - 189 High >/= 190 Very High	mg/dL
METHOD : CALCULATED			
NON HDL CHOLESTEROL	**230 High**	Desirable-Less than 130 Above Desirable-130-159 Borderline High-160-189 High-190-219 Very High- >or =220	mg/dL
METHOD : CALCULATED			
VERY LOW DENSITY LIPOPROTEIN	22.6	< or = 30	mg/dL
METHOD : CALCULATED			
CHOL/HDL RATIO	**6.8 High**	3.3 - 4.4: Low Risk 4.5 - 7.0: Average Risk 7.1 - 11.0: Moderate Risk >11.0: High Risk	
METHOD : CALCULATED			
LDL/HDL RATIO	**5.2 High**	0.5 - 3.0 Desirable/Low Risk 3.1 - 6.0 Borderline/Moderate Risk >6.0 High Risk	
METHOD : CALCULATED			

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Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A.CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >/= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors

Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

Interpretation of Diagnostic Tests by Jacques Wallach, 11th Ed.

Friedewald, W. T., Levy, R. I., & Fredrickson, D. S. (1972). "Estimation of the concentration of low-density lipoprotein cholesterol in plasma, without use of the preparative ultracentrifuge."Clinical Chemistry, 18(6), 499-502.

National Cholesterol Education Program (NCEP) Adult Treatment Panel III (ATP III). (2001).

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Third Report of the National Cholesterol Education Program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III) Final Report. Circulation, 106(25), 3143-3421.

American College of Cardiology (ACC) and American Heart Association (AHA) (2013). 2013 ACC/AHA Guideline on the Assessment of Cardiovascular Risk. Circulation, 129(25), S49-S73.

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Test Report Status **Final**	Results	Biological Reference Interval	Units
NEPHELOMETRY			
NEWGEN - HOME VISIT PACKAGE			
RF FACTOR (RHEUMATOID FACTOR QUANTITATIVE),SERUM			
RHEUMATOID FACTOR	<10.0	< 14.0	IU/mL

Interpretation(s)
RF FACTOR (RHEUMATOID FACTOR QUANTITATIVE),SERUM-This test is used for diagnosis of Rheumatoid arthritis (RA) in individuals with a suggestive clinical presentation.

Rheumatoid factor is an IgM autoantibody directed against the Fc portion of Immunoglobulin G (IgG) and is found in more than two-thirds of adults with Rheumatoid arthritis. Detection of RF is one of the criteria of the American Rheumatology Association (ARA) for the diagnosis of Rheumatoid arthritis.

The presence of Rheumatoid factor is of prognostic significance also, since patients with high titres tend to have more severe and progressive disease.

RF is also found in a number of other conditions such as Systemic lupus erythematosus, Sjogren's syndrome, chronic liver disease, hepatitis B. It plays an important role in differential diagnosis between RA and other rheumatic diseases.

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CLINICAL PATH - URINALYSIS

NEWGEN - HOME VISIT PACKAGE

PHYSICAL EXAMINATION, URINE

COLOR	PALE YELLOW		
APPEARANCE	CLEAR		

CHEMICAL EXAMINATION, URINE

PH	6.5	4.5 - 7.5	
METHOD : PH INDICATOR AND REFLECTANCE, SPECTROPHOTOMETRY			
SPECIFIC GRAVITY	1.005	1.005 - 1.030	
METHOD : PKA CHANGE WITH REFLECTANCE, SPECTROPHOTOMETRY			
PROTEIN	NOT DETECTED	NOT DETECTED	
METHOD : PROTEIN ERROR OF INDICATORS WITH REFLECTANCE, SPECTROPHOTOMETRY			
GLUCOSE	NOT DETECTED	NEGATIVE	
METHOD : GLUCOSE OXIDASE WITH REFLECTANCE, SPECTROPHOTOMETRY			
KETONES	NOT DETECTED	NOT DETECTED	
METHOD : ROTHERA'S WITH REFLECTANCE, SPECTROPHOTOMETRY			
BLOOD	NOT DETECTED	NEGATIVE	
METHOD : PEROXIDASE METHOD WITH REFLECTANCE, SPECTROPHOTOMETRY			
BILIRUBIN	NOT DETECTED	NOT DETECTED	
METHOD : DIAZOTIZED WITH REFLECTANCE, SPECTROPHOTOMETRY			
UROBILINOGEN	NORMAL	NORMAL	
METHOD : EHRLICH REACTION WITH REFLECTANCE, SPECTROPHOTOMETRY			
NITRITE	NOT DETECTED	NOT DETECTED	
METHOD : DIAZONIUM COMPOUND WITH REFLECTANCE, SPECTROPHOTOMETRY			
LEUKOCYTE ESTERASE	NOT DETECTED	NOT DETECTED	

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS	NOT DETECTED	0 - 2	/HPF
PUS CELL (WBCS)	0-1	0-5	/HPF
EPITHELIAL CELLS	1-2	0-5	/HPF
CASTS	NOT DETECTED		
CRYSTALS	NOT DETECTED		

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BACTERIA	NOT DETECTED	NOT DETECTED	
YEAST	NOT DETECTED	NOT DETECTED	

Interpretation(s)

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases
Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infectionwhen present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

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Henrys Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).

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SPECIALISED CHEMISTRY - HORMONE

NEWGEN - HOME VISIT PACKAGE

THYROID PANEL, SERUM

Test Report Status **Final**	Results	Biological Reference Interval	Units
T3 METHOD : ECLIA	110.30	Non-Pregnant Women 80.0 - 200.0 Pregnant Women 1st Trimester:105.0 - 230.0 2nd Trimester:129.0 - 262.0 3rd Trimester:135.0 - 262.0	ng/dL
T4 METHOD : ECLIA	**5.04 Low**	Non-Pregnant Women 5.10 - 14.10 Pregnant Women 1st Trimester: 7.33 - 14.80 2nd Trimester: 7.93 - 16.10 3rd Trimester: 6.95 - 15.70	µg/dL
TSH (ULTRASENSITIVE) METHOD : ECLIA	**18.360 High**	Non Pregnant Women 0.27 - 4.20 Pregnant Women (As per American Thyroid Association) 1st Trimester 0.100 - 2.500 2nd Trimester 0.200 - 3.000 3rd Trimester 0.300 - 3.000	µIU/mL

Interpretation(s)

Triiodothyronine T3 , **Thyroxine T4,** and **Thyroid Stimulating Hormone TSH** are thyroid hormones which affect almost every physiological process in the body, including growth, development, metabolism, body temperature, and heart rate.
Production of T3 and its prohormone thyroxine (T4) is activated by thyroid-stimulating hormone (TSH), which is released from the pituitary gland. Elevated concentrations of T3, and T4 in the blood inhibit the production of TSH.
Excessive secretion of thyroxine in the body is hyperthyroidism, and deficient secretion is called hypothyroidism.
In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hyperthyroidism, TSH levels are low.
Below mentioned are the guidelines for Pregnancy related reference ranges for Total T4, TSH & Total T3.Measurement of the serum TT3 level is a more sensitive test for the diagnosis of hyperthyroidism, and measurement of TT4 is more useful in the diagnosis of hypothyroidism.Most of the thyroid hormone in blood is bound to transport proteins. Only a very small fraction of the circulating hormone is free and biologically

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active. It is advisable to detect Free T3, FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4**.**

Sr. No.	TSH	Total T4	FT4	Total T3	Possible Conditions
1	High	Low	Low	Low	(1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment
2	High	Normal	Normal	Normal	(1)Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons.
3	Normal/Low	Low	Low	Low	(1) Secondary and Tertiary Hypothyroidism
4	Low	High	High	High	(1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3)Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy
5	Low	Normal	Normal	Normal	(1) Subclinical Hyperthyroidism
6	High	High	High	High	(1) TSH secreting pituitary adenoma (2) TRH secreting tumor
7	Low	Low	Low	Low	(1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism
8	Normal/Low	Normal	Normal	High	(1) T3 thyrotoxicosis (2) Non-Thyroidal illness
9	Low	High	High	Normal	(1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies

REF: 1. TIETZ Fundamentals of Clinical chemistry 2.Guidlines of the American Thyroid association duriing pregnancy and Postpartum, 2011.

TSH in pregnancy

There's reduction in both the lower and the upper limit of maternal TSH relative to the non-pregnant TSH reference range. This is because of elevated levels of serum hCG that directly stimulates the TSH receptor, thereby increasing thyroid hormone production. The largest decrease in serum TSH is observed during the first trimester. Thereafter, serum TSH and its reference range gradually increases in the second and third trimesters, but nonetheless remains lower than in non-pregnant women.

NOTE: It is advisable to detect Free T3,FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4.TSH is not affected by variation in thyroid - binding protein. TSH has a diurnal rhythm, with peaks at 2:00 - 4:00 a.m. And troughs at 5:00 - 6:00 p.m. With ultradian variations.

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SPECIALISED CHEMISTRY - VITAMIN

NEWGEN - HOME VISIT PACKAGE

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM

Test	Results	Biological Reference Interval	Units
25 - HYDROXYVITAMIN D METHOD : ECLIA	33.86	Deficiency: < 20.0 Insufficiency: 20.0 - < 30.0 Sufficiency: 30.0 -100.0 Toxicity > 100.0	ng/mL

VITAMIN B12(CYANOCOBALAMINE), SERUM

Test	Results	Biological Reference Interval	Units
VITAMIN B12 METHOD : ECLIA	201.2	197 - 771	pg/mL

Interpretation(s)
25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM- **Test description**
Vitamin D has anti-inflammatory and immune-modulating properties and it works towards the bones, teeth, intestines, immune system, pancreas, muscles and brain. It helps to maintain normal calcium and phosphate levels.Vitamin D is a fat-soluble vitamin. Also called as "Sunshine Vitamin".Two main forms as Cholecalciferol (vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight (Type B UV) exposure & Ergocalciferol (vitamin D2) present mainly in dietary sources.
Vit D25(OH)D deficiency is seen due to poor or inadequate sunlight exposure, Nutritional or dietary deficiency or fat malabsorption, Severe Hepatocellular disease, Secondary hyperparathyroidism, Hypocalcemia tetany which can cause involuntary contraction of muscles, leading to cramps and spasms, Rickets in children, Osteomalacia in adults- due to vitamin D deficiency mainly, Older adults- osteoporosis. (Increased risk of bone fractures)due to long-term effect of calcium and/or vitamin D deficiency, Other conditions that are precipitated by Vit D deficiency included increased cardiovascular risk, low immunity & chronic renal failure.
Elevated levels may be seen in patients taking supplements(hence recommended to repeat after 3 months for estimation of accurate levels), Vitamin D intoxication, sarcoidosis and malignancies containing non regulated 1-alpha hydroxylase in the lesion.
Recommendations
1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample 2.25(OH)D is the analyte of choice for determination of the Vitamin D status as it is the major storage & active form of Vitamin D and has longer half-life. 3. Kidney Disease Outcomes Quality Initiatives (KDOQI) and Kidney Disease Improving Global Outcomes (KDIGO) recommend activated vitamin D testing for CKD patients.
Note-Our Vitamin D assays is standardized to be in alignment with the ID-LC/MS/MS 25(OH)vitamin D Reference Method Procedure (RMP), the reference procedure for the Vitamin D Standardization Program (VDSP). The VDSP, a collaboration of the National Institutes of Health Office of Dietary Supplements, National Institute of Technology and Standards, Centers for Disease Control and Ghent University, is an initiative to standardize 25(OH)vitamin D measurement across methods.
Reference: 1.Wallach Interpretation of diagnostic test, 10th edition.
VITAMIN B12(CYANOCOBALAMINE), SERUM-**Test description**
1.Measures the amount of Vitamin B12/ Cyanocobalamin or Methyl cobalamin in blood.2. Done in Anemic conditions like Megaloblastic anemia, pernicious anemia, dietary folate deficiencies,3.Workup of neuropathies especially due to diabetes.4.Nerve health and it is monitored in treatment of nerve damage.5.Important vitamin for women of childbearing age and for older people.
1.Part of water-soluble B complex of vitamins. 2. It is essential in DNA synthesis, hematopoiesis & CNS integrity.3.Source for B12 is dietary foods like milk, yoghurt, eggs, meat, fortified cereals, bread. 4.Absorption depends on the HCl secreted by the stomach and occurs in intestines. 5. It is part of enterohepatic circulation, hence excreted in feces(approx. 0.1% per day)
Test interpretation

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Higher than normal levels are in patients on Vitamin supplements or patients with COPD, CRF, Diabetes, Liver cell damage, Obesity, Polycythemia.
Decreased levels seen in
Inflammatory bowel disease, Pernicious anemia - genetic deficiency of intrinsic factor - necessary for Vit B12 absorption, Strict vegetarianslead to sub-clinical B12 deficiency- high among elderly patients, Malabsorption due to gastrectomy, smoking, pregnancy, multiple myeloma & hemodialysis, Alcohol & drugs like amino salicylic acid, anticonvulsants, cholestyramine, cimetidine, Hyperthyroidism (High levels of thyroid), Seen in mothers of children with (NTD) Neural tube defects- hence fortification and supplements are advised in expecting mothers
Recommendations-1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample. 2. Vit B12 and Folic acid evaluated together in macrocytic anemias to avoid methyl folate trap. Carmel's composite criteria for inadequate Vit B12 status: Serum vitamin B12 < 148 pmol/L, or 148–258 pmol/L and MMA > 0.30μmol/L, or tHcy > 13 nmol/L (females) and >15 nmol/L (males).
Associated Test-Holo-TC: Marker of vitamin B12 status -specificity and sensitivity better than serum vitamin B12, hence recommended in boderline and deficient cases for confirmation.
References-O-Leary F, Samman S. Vitamin B12 in health and disease. Nutrients. 2010 Mar 2(3):299-316.

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